

Scaling-up Multi-Purpose Cash Assistance (MPCA) in Khartoum, Sudan

November 2025

Mission Statements

CORE (Community Organized Relief Effort)'s mission is to empower communities in and beyond crisis. CORE envisions a more equitable world in which underserved communities are prepared and can respond effectively to crises from within. CORE's Cash and Voucher Assistance vision specifically is to empower communities and meet their needs through timely, effective, and efficient assistance. To learn more, visit www.coreresponse.org and contact Mohamed Widaa, Sudan Response Director mohamed.widaa@coreresponse.org, and Tenzin Manell, Technical Director, Cash and Markets (tenzin.manell@coreresponse.org).

Sudanese Development Call Organization (NIDAA)'s mission is to serve, mobilize and motivate Sudanese society, longing for desirable change through training, advocacy and sustainable community development. To learn more visit www.nidaa.org and contact Sara Elgieli, Executive Manager (sara.elgieli@nidaa.org).

Acknowledgements

This learning brief was co-authored by Gersán Vásquez, Tenzin Manell and Omer Ahmed and made possible by the participants of CORE and NIDAA's programming as well as NIDAA team members Sara Elgieli, Hala Moawia, Ammar Abumedian, Nsraldin Adam and Mohamed Mustafa, Musaab Siddiq– and CORE team members Mohamed Widaa, Shaima Alnima, Nasreldin Jarout, Rasha Ahmed, Robert Danzi, Melanie Sandoval, Bethany Waggoner, Chantal Chastonay, and Angelina Kline.

This report was made possible by the generous support of the American people through the United States Agency for International Development (USAID) and through collaboration with Mercy Corps and the Cash Consortium of Sudan (CCS). The contents are the responsibility of CORE and do not necessarily reflect the views of USAID, the United States Government, the CCS or Mercy Corps.

1. Executive Summary

More than two years of escalating conflict in Sudan has pushed the country into alarming levels of food insecurity and critical loss of livelihoods. In response to the situation, CORE (Community Organized Relief Effort) and Sudanese Development Call Organization (NIDAA) implemented Multi-Purpose Cash Assistance (MPCA) in three localities of Khartoum State: Karari, and Omdurman, Sharganeel. The program, implemented in collaboration with the Cash Consortium of Sudan (CCS), aimed to increase households' capacity to meet essential needs in a dignified manner, support market functionality to serve people through the influx of MPCA, and mitigate the imminent protection risks faced amid the ongoing conflict.

Between July 2024 and June 2025, CORE and NIDAA reached 1,231 conflict-affected households classified as IPC 4 and 5 via bank transfer. The MPCA transfer value was aligned with the Cash Working Group (CWG) agreed Minimum Expenditure Basket (MEB) amount of SDG 280,000 (approximately US$108.50) per household per month, distributed in local currency. Assistance was provided in one tranche, totaling SDG 840,000 (US$325.50). While the majority of households required a proxy recipient to receive assistance, all respondents confirmed receiving the full transfer value.

Post-distribution monitoring findings demonstrated the following results:

- **Food Security**: Households experienced improvements in food security outcomes as the share of households with poor food consumption decreased by 73%.
- **Basic Needs**: MPCA enhanced households' capacity to meet basic needs, and on average, households allocated two-thirds of the assistance to food, medical expenses, debt repayment, and education.
- **Livelihood Coping Strategies**: MPCA reduced households' reliance on several negative coping mechanisms; however, increased reliance on social networks and migration was observed, a trend that warrants further analysis.
- **Well-being**: Households reported improved well-being outcomes, most notably through reduced stress.

The program demonstrates that scaling up MPCA remains a critical strategy to save lives, preserve dignity, and strengthen household resilience across Sudan. To enhance future program models, future interventions should:

- **Promote equitable partnerships** with local actors to advance localization and ensure sustainability.
- **Strengthen linkages between MPCA and livelihoods programming** to support pathways toward self-reliance.
- **Supporting recipients to open bank accounts,** promoting financial inclusion and reducing reliance on proxies.
- **Integrate gender and protection mainstreaming** to mitigate any unintended consequences and promote social inclusion and gender equality.

2. Introduction and Operational Context

Since April 2023, Sudan has spiraled into an unprecedented humanitarian crisis. More than two years of escalating conflict has pushed the country into alarming levels of food insecurity. According to the Integrated Food Security Phase Classification (IPC) system, which ranks food insecurity from Phase 1 (none) to Phase 5 (famine conditions), half of the Sudanese population (approximately 24.6 million people) are experiencing acute food insecurity (IPC Phase 3 or above). This represents an increase of 3.5 million people compared to the previous analysis in October 2024 and includes five areas where famine has been confirmed, as well as five more at imminent risk (Integrated Food Security Phase Classification [IPC], 2025). The situation is especially severe for female-headed households which experience food insecurity at twice the rate of male-headed households (United Nations Office for the Coordination of Humanitarian Affairs [OCHA], 2024). In Khartoum State, long the epicenter of the conflict, 75% of the population face high levels of food insecurity (Integrated Food Security Phase Classification [IPC], 2025).

Severe protection violations related to the conflict have triggered mass displacement across the country. The latest figures estimate more than 13 million people have been forced to flee their homes, either within or across borders, making this displacement crisis globally (International Organization for Migration [IOM], 2025). For the first time since the conflict began, the number of internally displaced persons (IDPs) has begun to decline. This recent shift is due to an influx of returnees, many of whom had been displaced out of Khartoum State. Most returnees have resettled in Omdurman-Karari locality.

The impact of the conflict extends beyond food insecurity and displacement. It has caused extensive damage including the destruction of local infrastructure, collapse of essential services, and loss of livelihoods, leaving millions without access to basic needs, deepening economic hardship, and exacerbating a severe public health crisis. As a result, the extreme poverty rate is expected to more than double from 33% in 2022 to 71% of the population in 2025, according to the World Bank.

In response, Community Organized Relief Effort (CORE) and its national partner. Sudanese Development Call Organization (NIDAA) implemented a Multi-Purpose Cash Assistance (MPCA) program targeting 1,231 hard-to-reach conflict-affected households in Karari, Omdurman, and Sharganeel in Khartoum State. The program, implemented in collaboration with the Cash Consortium of Sudan (CCS), demonstrated measurable improvements in food security, reduced reliance on negative coping strategies, enhanced household well-being, and strengthened capacity to meet basic needs. This learning brief presents key findings from 290 Post Distribution Monitoring surveys (32% female) and outlines lessons learned and recommendations to inform future at-scale MPCA responses in Sudan's evolving humanitarian context.

3. Program Model

Program Objective and Response Modality

In coordination with other members of the Cash Working Group (CWG), and in alignment with the national and sub-national CWGs in Sudan, CORE and NIDAA delivered cash assistance based on inter-agency cash

feasibility assessments. The program aimed to increase households' capacity to meet essential needs in a dignified manner, support local market functionality through cash injections, and mitigate imminent protection risks amid the ongoing conflict.
Between July 2024 and June 2025 CORE and NIDAA reached 1,231 conflict-affected households[1] (24% female-led households) with MPCA across three localities of Khartoum State (see **Figure 1)**: Karari (Al-Iscan95), Omdurman (Almawrda), and Sharganeel (Almaygoma). These localities were prioritized due to their high levels of humanitarian needs, limited accessibility, and absence of other actors providing MPCA at the time of implementation.

MPCA was delivered alongside complementary health, nutrition, and protection support through referral pathways. Together, CORE and NIDAA, in coordination and collaboration with the CCS and CWG, contributed to strengthening and harmonizing the humanitarian response efforts across Khartoum State.



Figure (1) Location of Program by Share of Participants in Khartoum State, Sudan

Transfer Value, Frequency and Duration

The MPCA transfer value aligned with the CWG agreed MEB amount of SDG 280,000 (US$108.50) per household per month, distributed in local currency. Assistance was provided to cover three months of basic needs in one tranche, totaling SDG 840,000 (US$325.50). Depending on the security context, CCS members distributed assistance monthly for a period of three months or in one tranche. Due to heightened insecurity in Khartoum for both staff and recipients, CORE and NIDAA opted to deliver one tranche equivalent to three months of assistance.

[1] CORE's original target was 1,118 households, which it was able to exceed due to an underspend related to currency conversion from Euro (EUR) to Sudanese Pounds (SDG).

Delivery Mechanism

MPCA was delivered in partnership with RedRose and Cashi, and distributed via bank transfers, specifically Bankak. To maximize accountability, efficiency and safety for staff and recipients, CORE and NIDAA adopted a fully digital transfer system during program design, drawing on lessons learned from a previous self-funded hybrid (physical and digital) MPCA distribution in Khartoum State. During implementation, the Humanitarian Aid Commission (HAC) announced that all humanitarian cash transfers should be limited to digital distribution; having already established this approach, CORE was able to proceed without delays. Close coordination with the Community Committees helped ensure distributions were conducted safely for both recipients and project staff despite the volatile security situation.

Targeting, registration, verification, enrolment and proxies

In alignment with the CCS members, CORE and NIDAA coordinated with HAC to adopt a blanket targeting approach in IPC Phase4 areas of Khartoum State, given the prevailing security constraints at the time of implementation. NIDAA led community coordination with existing Community Committees and local leaders to ensure that households in these hard-to-reach areas were appropriately identified for assistance. All households on the Community Committee lists were first registered in hardcopy and subsequently digitized in the CCS CommCare system and deduplicated. For households without access to a bank account, proxy recipients were coordinated during registration by NIDAA staff and Community Committees. The majority of households (59%) required a proxy recipient arrangement. In these cases delegation forms were completed and signed by both the intended recipient and the proxy recipient. The Community Committees played a critical in the registration process, providing localized security analysis and facilitating safe access for staff and participants throughout registration.

Referrals

CORE engaged with the CCS and Community Committees to establish and activate referral pathways for vulnerable individuals identified during registration and distribution. While the CCS referral system was being operationalized, CORE and NIDAA facilitated referrals through existing projects for health-related needs and critical pregnancy cases. All referrals involved women and children and originated from Shargalneel, an area under siege at the time, where access to basic services was extremely limited.

Despite these efforts, referral activities faced significant constraints, as most services in the area were non-operational or severely limited. Escalating hostilities and ongoing clashes within intervention areas further restricted safe access to complementary services. In addition, the absence of a standardized referral system and designated focal points among partners created coordination challenges. In response, CORE and NIDAA established ad hoc referral mechanisms to ensure that urgent cases received at least minimal assistance.

Community Feedback and Complaint Mechanisms (CFMs)

Various Community Feedback and Complaint Mechanisms (CFMs) were deployed during the program, including WhatsApp numbers for designated CORE and NIDAA staff, designated CORE and NIDAA protection focal points, and the Community Committees. Community Committees played an active role in providing regular feedback through a dedicated focal point for recipients.

4. Methodology, Data, and Limitations

PDM Design and Sampling Strategy

Before MPCA distribution, CORE collected demographic data for all 1,231 cash recipients using CommCARE. CORE also conducted a baseline analysis with a representative random sample of 295 individuals. To measure the longitudinal impact of MPCA, a PDM survey was conducted. Data was collected using a structured questionnaire administered through KoboToolbox. The survey included mostly close-ended questions. The unit of analysis was household, and the survey covered seven components: (1) Demographic Information; (2) MPCA Expenditure; (3) MPCA Effectiveness; (4) Food Consumption Score (FCS); (5) Livelihood Coping Strategies; (6) Well-being; and (7) Accountability.



The data collection process was conducted over a seven-day period between the end of May and the beginning of June 2025, using a simple random sampling method was implemented. The sample size was estimated based on research standards. A total of 294 PDM surveys were collected. Following the data cleaning to remove incomplete submissions, a final representative sample of 290 valid responses was retained, representing 24% of all MPCA recipients. Before data collection, CORE staff and local partner NIDAA, in coordination with local authorities and Community Committees, conducted a community meeting with participants to explain the purpose of the survey, how the data would be used, and their right to withdraw at any point. A total of seven enumerators (three female) were hired and received in-person training. CORE and NIDAA prioritized gender parity to ensure female interviewees felt comfortable completing the survey.

Demographic Characteristics

The average respondent was female (68%) and 50 years old, with ages ranging from 18 to 84. At baseline, 12% were living with a disability. The majority of MPCA recipients (85%) were married. and 17% reported living in a polyamorous household. The typical household consisted of four members, which is smaller than the national pre-conflict average of six people per household. This result is likely due to the effects of mass displacement, as all respondents self-identified as IDPs. Almost one in 10 households included a pregnant or lactating woman.

Limitations

Both a PDM survey and endline assessment were planned, but due to changes in funding allocations, it was not feasible to complete the endline. The PDM data collection process covered only participants from Omdurman due to security concerns, which limits the analysis of the report and the generalizability of findings to other affected areas of Khartoum State where the program provided MPCA. In addition, the return of displaced participants, many of whom were temporarily hosted in CORE's operational areas, limited the ability to conduct house-to-house interviews with some MPCA participants. Despite these limitations, the data provides insights into the outcomes of conflict-affected households reached through the MPCA program and provides insights to inform future programming.

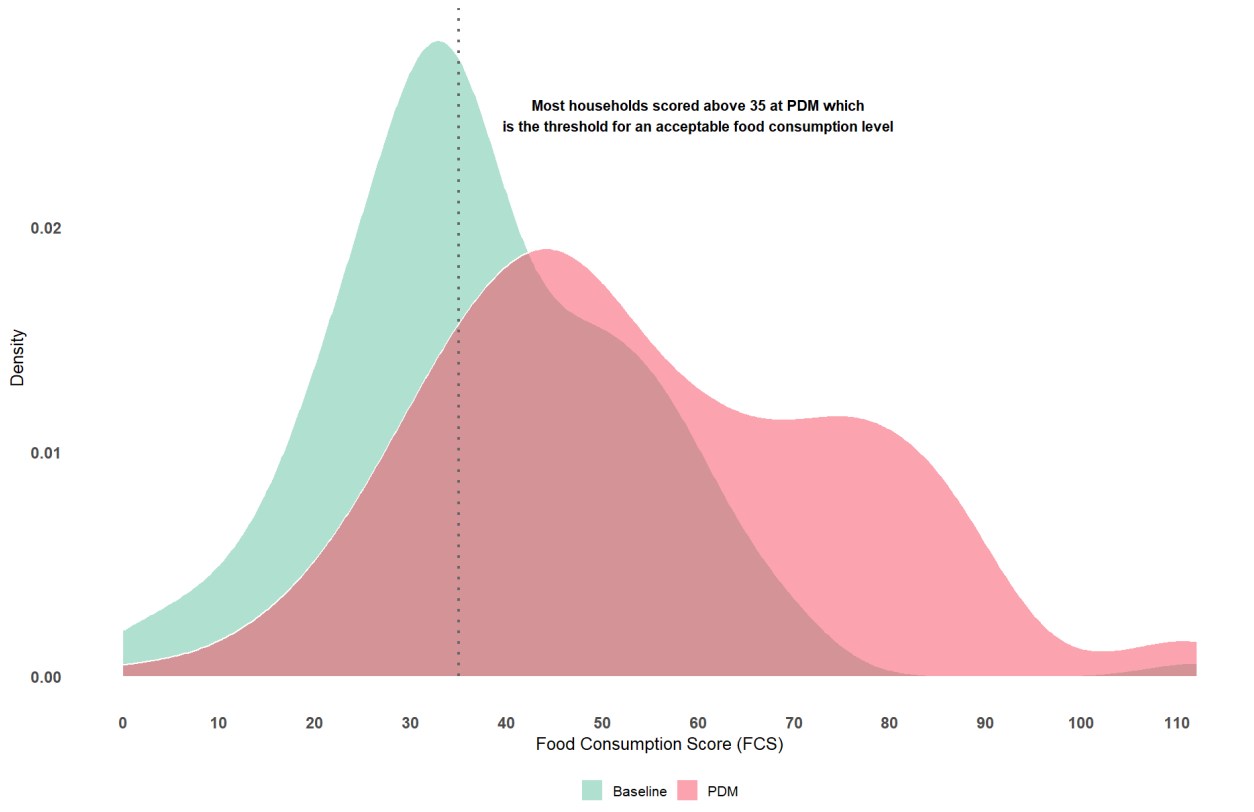
5. Findings

Data gathered from traders and local population estimates informed the consumption projections presented below.

Nutrition: Food Consumption Score (FCS)

The FCS measures dietary quality and the frequency of food group consumption over a seven-day reference period. According to WFP's FCS classification, households were categorized into three groups: poor FCS (0-21), borderline FCS (21.5-35), or acceptable FCS (more than 35). Both baseline and PDM data were captured using this approach. The results suggest that MPCA contributed to improved household food security outcomes[2]. **Figure 2** shows that the distribution of the FCS at PDM shifted to the right, as compared to baseline. This shift suggests an overall increase in average scores for the majority of participants. Following receipt of MPCA, most households scored above 35, which is the threshold for an acceptable food consumption level. The PDM distribution curve also appears wider than the baseline curve, suggesting greater variability in food security outcomes, as some households achieved scores between 60 and 75, a range that was uncommon at baseline.

Figure (2) Kernel Density Estimate of the FCS Distribution of Households at Baseline and PDM

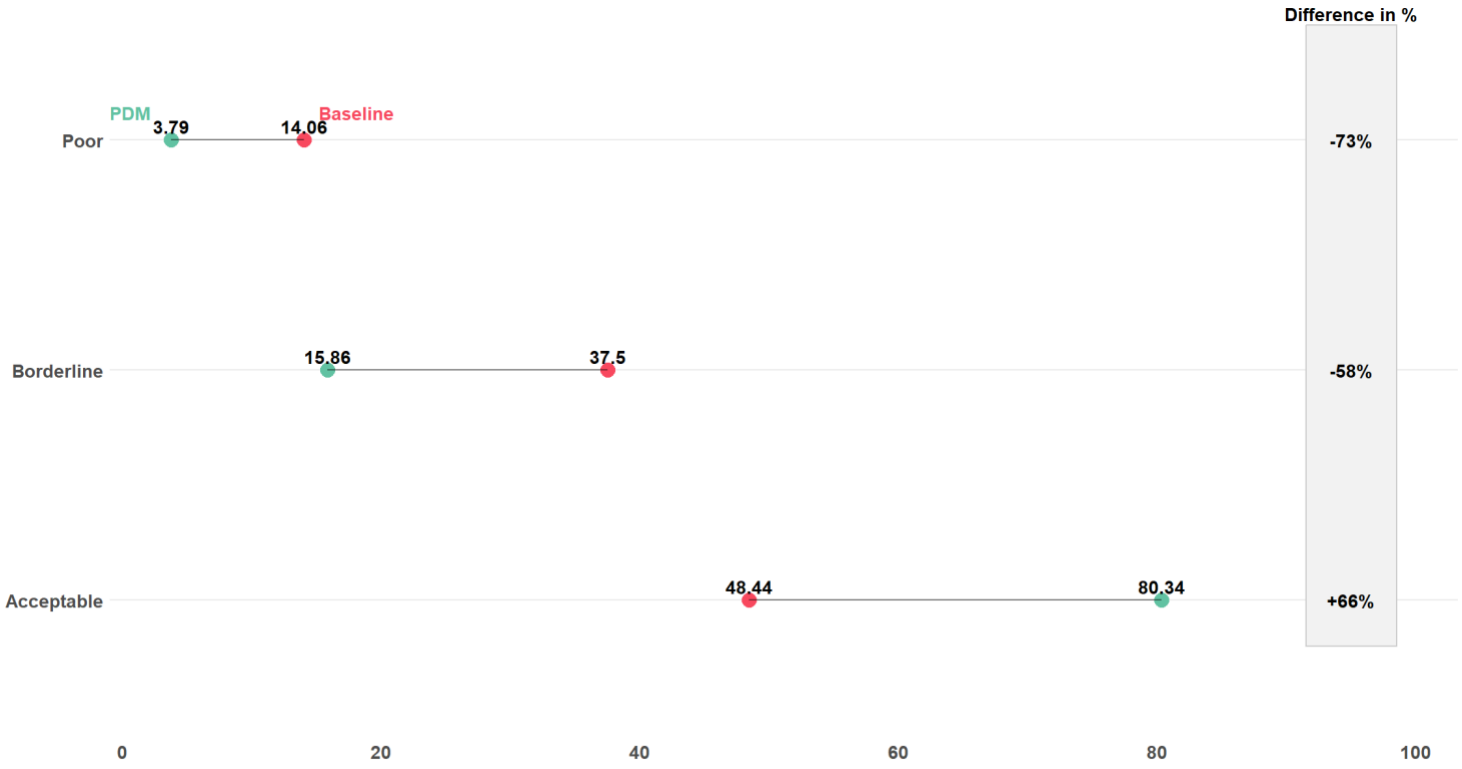


Source: Authors' elaboration using baseline (n=128) and PDM data (n=290).
Note: The height of the curve is scaled such that the area under the curve equals one.

[2] The survey design did not allow for a causal impact analysis.

As shown in **Figure 2**, at baseline, close to 20% of households were classified as having poor food consumption. Being in the poor FCS category implies that these households were unable to meet their basic food needs. Most households in this category were single female-headed households. By the end of the program, the proportion of households in poor food consumption declined by 73%. In addition, the share with borderline food consumption decreased by 58% while those in acceptable food consumption increased almost double at PDM. Disaggregated analysis shows that female- and older person-headed households also experienced improvements in food outcomes. However, these differences were not statistically significant. In other words, age and sex were not associated with differences in FCS within this sample.

Figure (3) FCS Categories at Baseline and PDM, in percentages



Source: Authors' elaboration using baseline (n=128) and PDM data (n=290).
CORE used the recommended cut-off points: Poor (FCS < 21), Borderline (21.5-35), and Acceptable (>35).

On average, households gained 1 FCS point for every SDG 48,735 (US$19) transferred, or 5.3 FCS points for every SDG 256,000 (US$100) distributed per household. Based on this relationship, it would cost about US$285, as a lump sum, to lift a household from poor to acceptable FCS, assuming a starting score of 21 and an endpoint of 36[3].

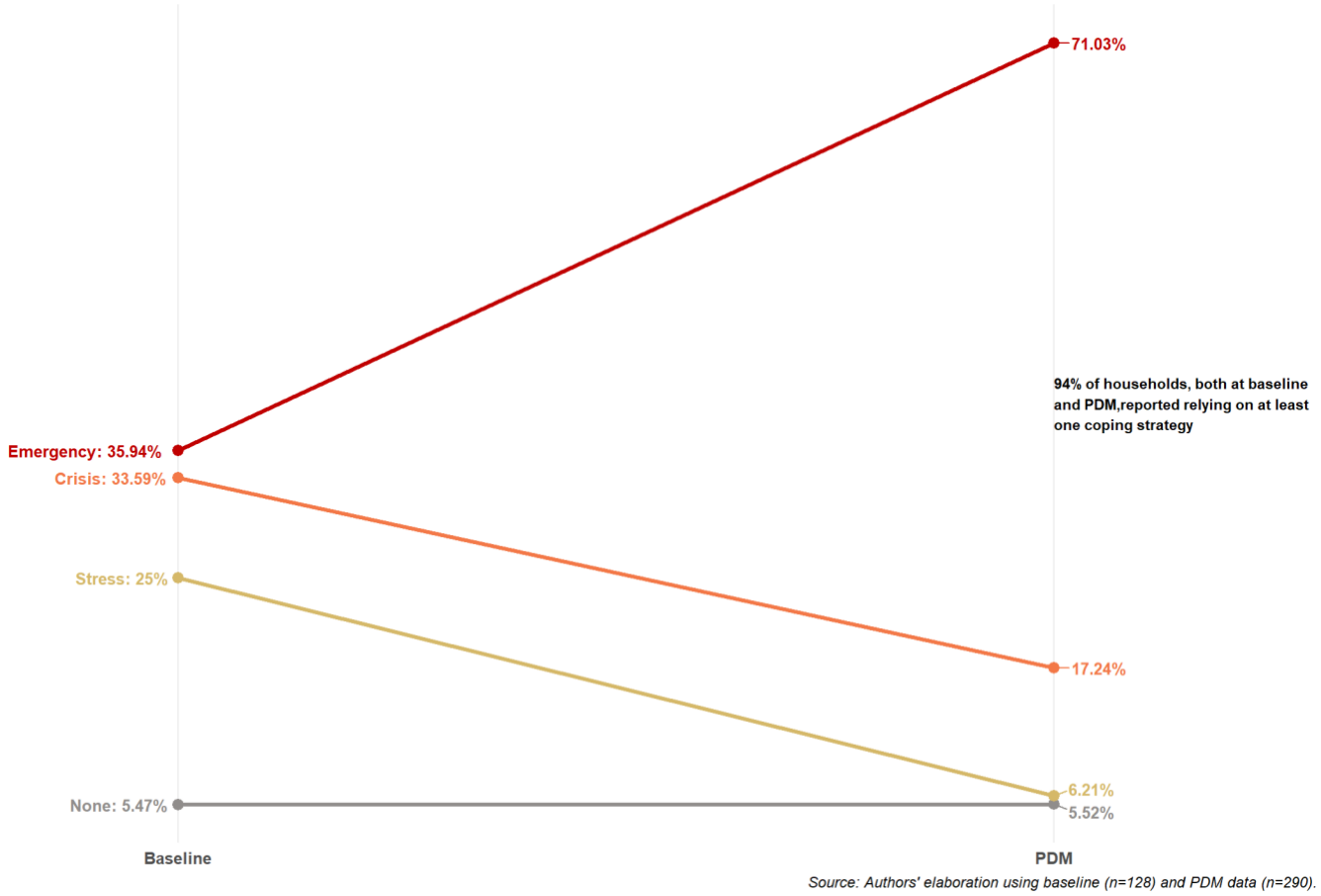
Livelihood Coping Strategy Index (LCSI)

The LCSI measures households' coping capacity in response to shocks. The LCSI was grouped into three categories based on severity: stress, crisis, and emergency. Respondents were asked whether they had resorted to any of the ten pre-defined negative livelihoods coping strategies within the last thirty days. As

[3] The analysis didn't include operation cost or cash transfer fees.

shown in Figure 4, 94% of households, both at baseline and PDM, reported relying on at least one coping strategy. What changed over time was the type of strategies used. Overall, the prevalence of crisis and stress strategies decreased, while emergency strategies increased. These categories are discussed further below.

Figure (4) Share of Households Relying on Different Coping Strategies at Baseline and PDM



Source: Authors' elaboration using baseline (n=128) and PDM data (n=290).

No Negative Coping Strategies

The proportion of households that did not need to rely on any negative coping strategies remained unchanged at 6% at both baseline and PDM. This result suggests ongoing vulnerability in households' ability to manage shocks. A gender comparison shows that all female-headed households reported using at least one coping strategy, compared to 7% of male-headed households who reported not relying on any. No statistically significant differences were observed between younger- and older-headed households.

Stress strategies

As shown in **Figure 5**, the percentage of households reporting reliance on stress strategies declined fourfold at PDM. Disaggregated results after the intervention show there was no statistically significant difference in the use of stress coping strategies between male- and female-headed households, nor between younger- and older-headed households. At the individual level, reliance on three out of the four coping strategies decreased: purchasing food on credit or borrowing food (-43.35%), borrowing cash (-54%), spending savings (-168.35%). There was, however, an increase in the share of households that sold household assets or goods (+21.07%).

Crisis strategies

The percentage of households reporting reliance on crisis coping strategies decreased twofold by PDM (see **Figure 5**). Disaggregated results show that female-headed households were 1.5 times more likely to rely on crisis strategies than male head of households. When looking at the analysis results by the individual coping strategies, there were a decrease in two out of the three strategies: withdrawing children from school[4] (-47.87%) and selling productive assets or means of transport (-63.29%).

There was an increase in households reporting reduced expenditure on essential non-food items (+19.11%). The concurrent increase in households selling non-productive assets/goods and the decrease in the sale of productive assets may have mitigated the need to sell productive assets. However, given that the level of needs remains high, households may have resorted to selling readily tradable household goods, in order to meet immediate consumption requirements.

Emergency strategies

In contrast, reliance on emergency coping strategies doubled at PDM (see **Figure 5**). Disaggregated results show that older-headed households were 2.5 times more likely than younger-headed households to adopt these strategies after the intervention. The increase was driven by an increase of the share of households who relied on direct support from friends or family (+137.17%) and those reporting at least one household member migrated (+43.61%), the former likely explained by the overall context of increased forced displacement. The rise in household-level emergency strategies may appear counterintuitive given the decline in stress and crisis strategies.

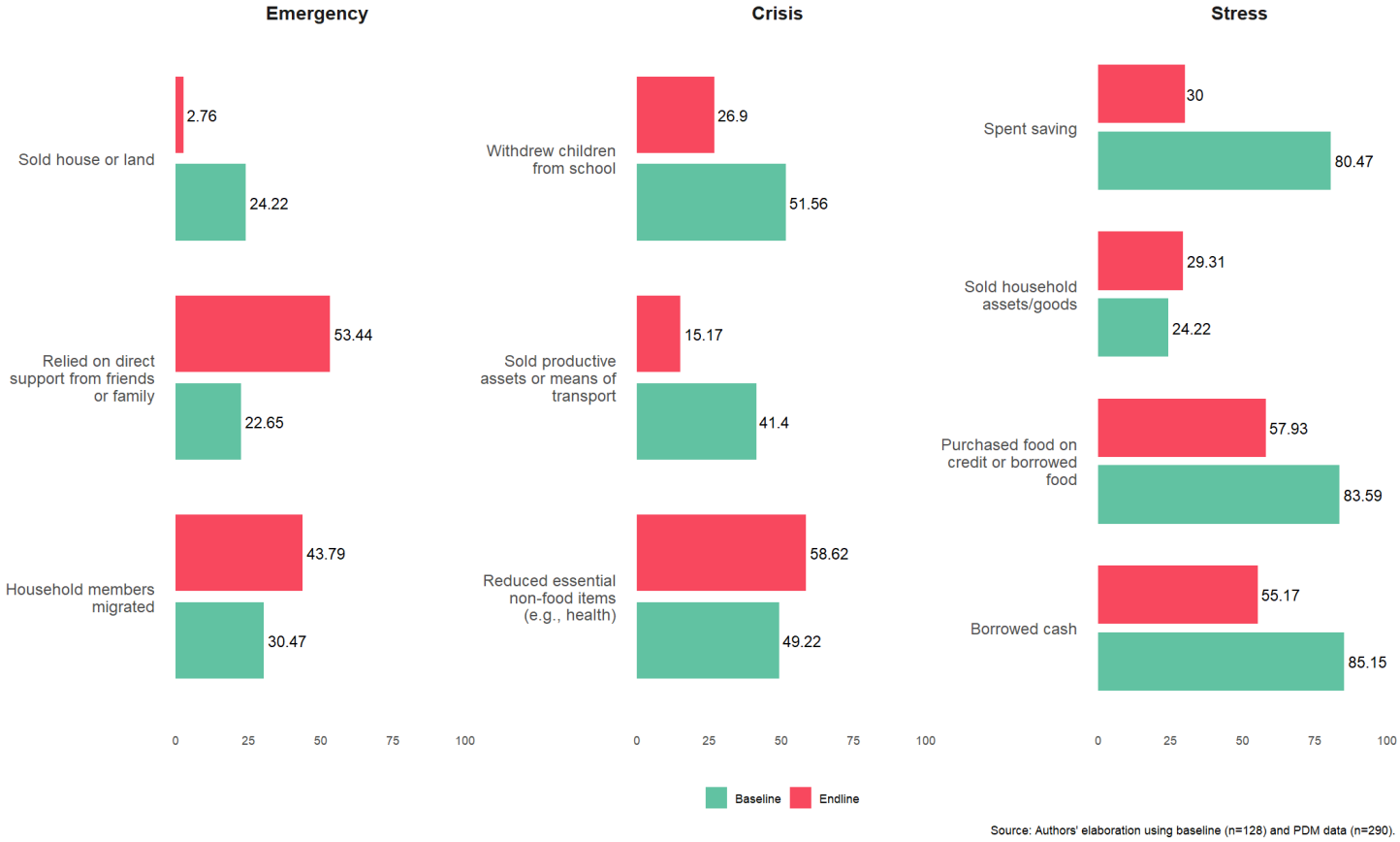
A potential explanation is that the MPCA may have alleviated the need to adopt moderate coping mechanisms. Nonetheless, MPCA alone may have been insufficient to offset the effects of structural shocks such as hyperinflation and prolonged conflict. It is also possible that over time, households transitioned from less severe coping behaviors (e.g., spending savings that were fully depleted) to more severe strategies, such as relying on migration[5] or support from their networks. In addition, the increased reliance on families and friends may also suggest a strengthening of informal support systems and social networks, which could indicate distress but also community-level resilience.



[4] The results are limited to locations where schools were in operation.

[5] MPCA might have supported families to send away either a most vulnerable or key breadwinner by reducing the cost to migrate. However, it is not possible to test this hypothesis as it was outside of the scope of the PDM.

Figure (5) Share of Households Relying on Different Coping Strategies by Severity at Baseline and PDM



Source: Authors' elaboration using baseline (n=128) and PDM data (n=290).

Well-being

Well-being was measured using an index[6] of four dimensions: (i) ability of the household to meet basic needs, (ii) improved living conditions, (iii) reduced financial burden, and (iv) reduced feelings of stress[7]. **Figure 6** shows that MPCA contributed to short-term improvements in participants' well-being outcomes. Close to 80% of participants scored in the *high* category on the well-being index after receiving MPCA. The results were higher for female participants (92.41%) and adults aged 65 and above (93.1%). The four dimensions are discussed in more detail below.

Reduced feelings of stress

Two-thirds of MPCA recipients (66%) reported that MPCA enabled them to *significantly* reduce their feeling of stress. Although women and older adults also reported improvements in this dimension after PDM, the differences between men and women, and older and younger adults, were not statistically significant.

[6] The index was based on four questions scored on a 4-point Likert scale (3=Significantly, 2=Moderately, 1=Slightly, 0=Not at all). Scores (0–12) were aggregated and normalized to a 0–100 scale; values ≥66.6% were classified as high well-being.
[7] CORE conducted the coefficient of reliability test Cronbach's Alpha. The resulting coefficient was **0.8712**, suggesting that the variables had relatively high internal consistency.

Allowed your household to meet basic needs

Sixty-two percent (62%) of participants reported that MPCA *significantly* improved their household's ability to meet basic needs. On average, younger and male participants were more likely to experience higher levels of well-being outcomes in this dimension compared to older and female participants. This finding is consistent with the results from the coping strategies analysis (section 4.2), as female and older individuals were more likely to rely on emergency coping strategies.

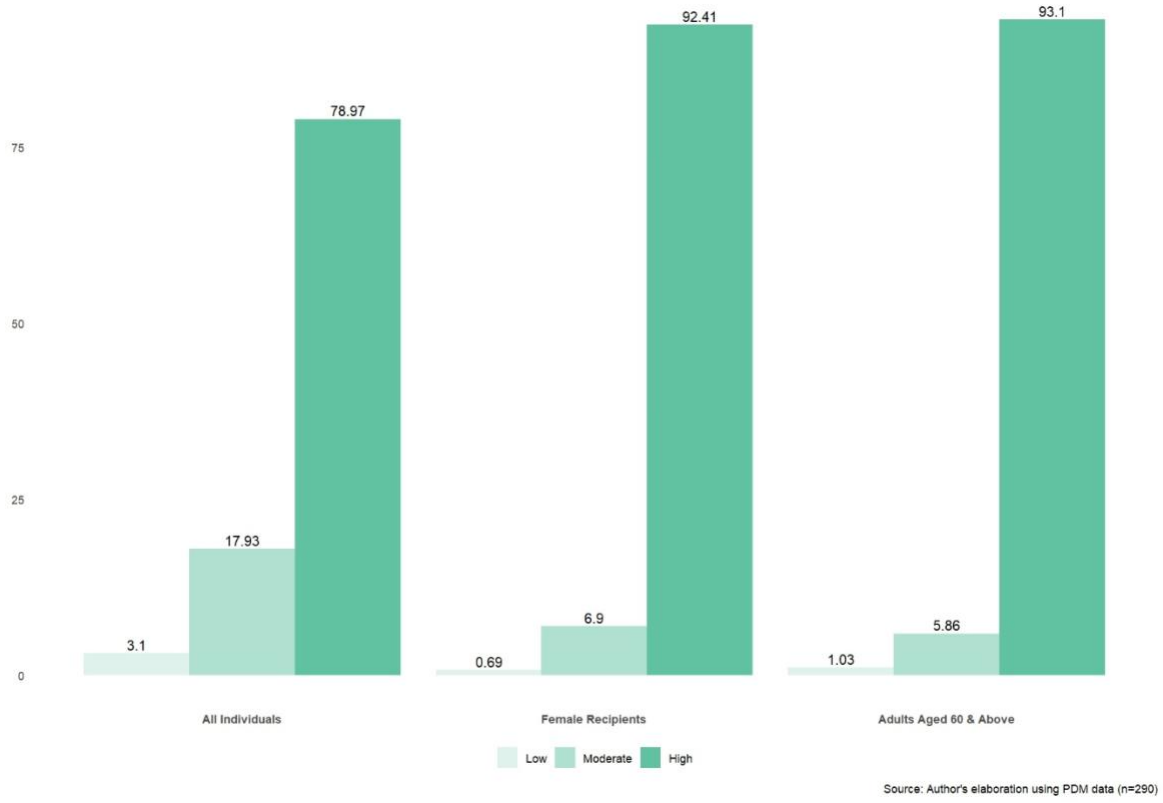
Improved your living condition

Six in ten participants stated that MPCA *significantly* improved their living conditions[8]. Persons living with disabilities and older adults experienced improvements in living conditions; however, these differences were not statistically significant.

Reduced the financial burden of your household

Fifty-nine percent (59%) of respondents reported that MPCA *significantly* reduced their household's financial burden. Female-headed households experienced a higher reduction of financial burden compared to male-headed households, while younger-headed households and people not living with disabilities were more likely to report improvements than older-headed households and people living with disabilities, respectively. However, these differences were not statistically significant.

Figure (6) Well-being Index by Categories and Selected Demographic Groups, In Percentages



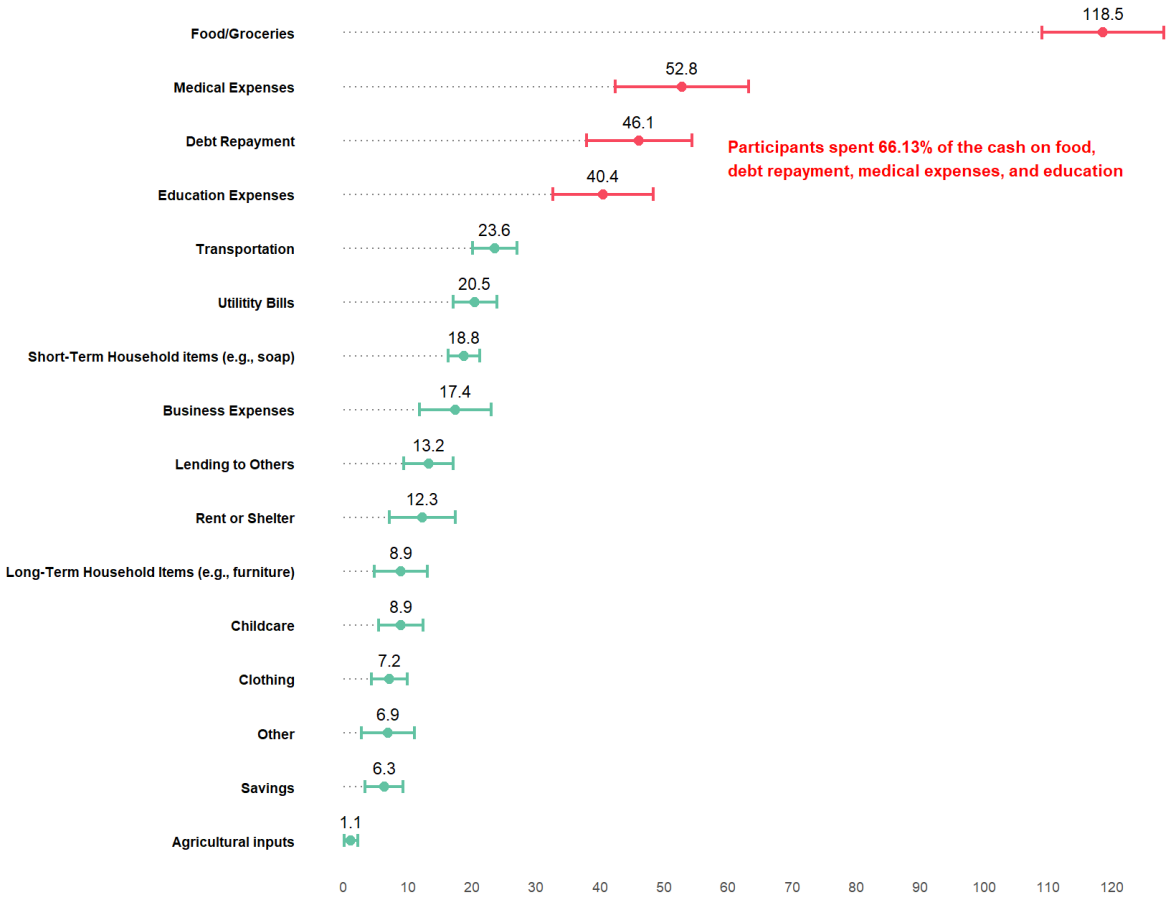
[8] Female respondents reported lower results compare to male respondents, but the results were not statistically significant either.

MPCA Expenditure and Decision-Making

Spending patterns suggest that households used MPCA to meet their immediate basic needs. On average, participants allocated two-thirds of their MPCA to four expenditure categories: food and/or groceries, medical expenses, debt repayment, and education (see Figure 7). The results are consistent with the improvements observed in food security outcomes (Section 4.1). Expenditure patterns were, on average, similar across age and sex. The only exceptions were business investments and childcare expenses. Younger participants invested more in businesses activities (US$20.56) than older participants9 (US$8.58), whereas female recipients spent more (US$11.12) on childcare than male recipients (US$4.27). While most recipients were able to purchase all the items they needed, 30% reported that they could not buy certain food items (e.g., milk, fruits), medications, and furnishings. Responses regarding decision-making patterns varied: 87% of respondents reported that they themselves made decisions, while the remaining 13% reported decisions being led by other family members, most often their partners. The results were consistent by sex. For most households (93%), MPCA did not change existing decision-making dynamics. However, among those who reported changes (7%), there was an increase in joint decision-making between men and women.

Figure (7) *MPCA Expenditure by Categories in Nominal US$*

Shown are the mean answers and the 95% confidence intervals



Source: Author's elaboration using PDM data (n=290).
Note: Paralell exchange rate US$ 1 = SDG 2,565 as of June 2025 from the WFP. Survey prices were asked in SDG.

[9] There was weak evidence that younger participants spent more on transportation and rent/shelter compared to older participants. However, the differences were only significant at 10% levels.

Modality, Transfer Value, Timing, and Frequency

Response Modality and Delivery Mechanism

At baseline, all program participants indicated that bank transfer was their preferred delivery mechanism. At PDM, the preference for bank transfers remained high (96.94%). A small share of respondents preferred other modalities: 2.04% favored multi-modal support combining cash and in-kind assistance, and 1.02% preferred electronic transfers through the Cashi Wallet network.

Despite the preference for bank transfers, many participants did not have their own active bank accounts. Most respondents (59%) reported receiving assistance through authorized and designated proxies' accounts after they signed a delegation letter–proxies were most often members of their immediate or extended families as well as neighbors—while the remaining 41% received assistance directly into their own accounts. All participants that used proxy accounts reported receiving MPCA and, overall, experienced no major issues. The majority reported being satisfied with the process (97%), maintaining smooth communication with their delegated proxy recipient (93%), and receiving MPCA on the same day as communicated by CORE and NIDAA[10]. Despite the successes of proxy transfers, the high reliance on proxies underscores the need for support to open bank accounts. The civil registration system has rendered many program participants unable to renew their identification documents (IDs), thereby preventing them from having valid IDs to open bank accounts.

Transfer Value, Timing and Frequency

All respondents confirmed receiving the full transfer value of SDG 840,000 (US$325.50). Information about the transfer was communicated primarily during community meetings (45.19%) and through local representatives (31.85%). 90% of respondents expressed that the transfer amount was equal to what the program team had communicated, while the remaining 10% reported that they were not informed in advance. The majority (93%) reported that they received MPCA on the expected day, and the remaining 7% were unsure. Ninety-four percent (94%) of respondents preferred to receive MPCA as a single tranche, as defined in the program design. Only 5% preferred multiple tranches[11], and 1% reported no preference. Participants who preferred a single tranche explained that phased disbursement would reduce their ability to address urgent needs and risk additional value loss due to currency depreciation and high inflation.

Protection Mainstreaming and Accountability to Affected Population

Safety

Most participants reported feeling safe during the registration, while accessing MPCA, and when spending MPCA. As shown in **Table 1**, eight out of ten respondents felt safe during registration, and 85% felt safe while receiving MPCA and spending MPCA. Although the majority felt safe, between 15% to 20% reported feeling a bit unsafe or very unsafe. Older respondents reported relatively lower levels of safety compared to younger individuals, but the differences were not statistically significant. The CCS PDM instrument did not collect information on the specific reasons for feeling unsafe.

[10] 16% received it between 1 to 2 days, 7% between 3 to 5 days, and 4% took more than 5 days.
[11] Women (8%) were more likely than men (5%) to prefer multiple tranches but the differences are only significant at 10% levels.

Table (1) Share of Participants Expressing Safety Levels at Different Moments of the MPCA Assistance

	All Participants		Female Participants		Old Participants	
	Very Safe or Safe	**A bit Unsafe or Very Unsafe**	**Very Safe or Safe**	**A bit Unsafe or Very Unsafe**	**Very Safe or Safe**	**A bit Unsafe or Very Unsafe**
Safety During Registration	**79.32%**	20.68%	**85.71%**	14.29%	**76%**	24%
Safety Receiving MPCA	**85.82%**	14.18%	**92.55%**	7.45%	**81.58%**	18.42%
Safey Spending MPCA	**85.14%**	14.86%	**90.32%**	9.68%	**81.58%**	18.42%

***Source**: Author's elaboration using PDM data (n=290).*

Community Feedback Mechanisms (CFMs)

The functionality and reach of CFMs were constrained by widespread internet and mobile network disruptions, which restricted the use of WhatsApp, e-mail, and hotlines. In addition, insecurity and heightened conflict reduced opportunities for in-person engagement with protection focal points and Community Committees. When asked about preferred channels for future feedback and complaints, respondents preferred a hotline (80.67%) and meeting with local representatives (9.66%)[12].

Respect, Needs Covered, Targeting, and Satisfaction

PDM respondents were asked about their perceptions of respect, how well their needs were covered, the targeting approach, and overall satisfaction. These are discussed below.

Respect: Ninety-eight percent (98%) of respondents reported being treated with respect by CORE and NIDAA staff during the full program cycle, including during assistance delivery. The remaining 2% did not elaborate further on the reasons.

Needs covered: Ninety percent (90%) felt their needs were considered. Among the 10% who disagreed, respondents expressed dissatisfaction with the transfer value, which they felt was insufficient due to rising prices and difficulties affording medicines. Disaggregated analysis indicated that female and older respondents were more likely to feel that their needs were not considered. Nonetheless, the differences were not statistically significant.

Targeting: Perceptions on targeting were mixed; while 40% of respondents were satisfied with the selection process, another 40% reported knowing someone in need of MPCA who was not included in the program. It was unclear from the phrasing of the question whether responses referred to individuals excluded from assistance within the target geography or not reached by humanitarian assistance in general.

Satisfaction: Nearly all participants reported being "very satisfied" or "satisfied" with the overall program, with only two individuals expressing neutral opinions. Suggestions for improvement included increasing the transfer value, adjusting the transfer amounts based on household size, providing small business grants alongside MPCA, and expanding assistance coverage to reach more families.

[12] The remaining ones were SMS (2.2%) and Email (1.26%).

Transparency, Fairness and Safeguarding

To ensure issues related to transparency, fairness and safeguarding within MPCA could be identified and addressed, participants were asked during the PDM whether anyone had requested a fee, gift, tip, service, or favor in exchange for assistance. All but one respondent answered no such incident. The individual who did report an incident stated that someone—who was not a program staff, community organizer, or delegate—had asked for a bribe, which she refused. The participant declined to provide additional information.

6. Learnings and Recommendations

Category	Finding/Learning	Recommendation
Response Modality and Delivery Mechanism	During registration, all program participants indicated that cash assistance was their preferred response modality, with 96.94% preferring bank transfers. However, the majority of targeted participants relied on a proxy because they did not have their own bank account. Close coordination with RedRose and Cashi throughout the CVA program cycle enabled timely distribution.	Continue to prioritize cash assistance over other response modalities paired with support to recipients in obtaining identifying documents and opening bank accounts. This may include guidance and accompaniment during the registration process to reduce reliance on proxies. Advocacy with relevant stakeholders, including HAC, the Bank of Khartoum, the Civil Registry, and other actors, can facilitate the renewal/issuance of identity documents and the opening of new accounts. Referrals from MPCA to renewal and issuance services are key. This will also contribute to greater financial inclusion among recipients of MPCA. Looking ahead, integrating MPCA assistance with livelihoods recovery initiatives will further strengthen household resilience and self-reliance.
Transfer value	All respondents expressed satisfaction with the transfer value, indicating they were “very satisfied.” This suggests that the transfer value was aligned with local market realities and that the targeting and planning processes were responsive to participants’ needs and priorities.	Continue to align with CWG-established and CCS-endorsed MEB transfer value. Expand local market monitoring activities to assess price fluctuations and identify potential market distortions.
Transfer frequency and duration	The majority of recipients preferred to receive MPCA as a single tranche equivalent to three months of assistance.	Where preferred, and when security conditions require, continue to deliver assistance in one tranche.
Targeting	The targeting approach was developed through close coordination by CORE and NIDAA with local authorities, and Community Committees, and targeted	While the blanket targeting approach contributes to reducing potential community tensions, consider adopting vulnerability-based targeting that incorporates additional variables beyond IPC level, using pre-set

	vulnerable households in IPC Phase 4 and above.	criteria to better reach the most vulnerable households based on the context in the targeted location. The pre-set criteria should be discussed with Community Committees and the CCS.
CFM	Program participants preferred using an anonymous hotline to report issues or provide feedback. However, frequent power outages and poor network coverage in Sudan limited consistent access, while other CFM options such as community leaders, local representatives, complaint boxes, SMS messages, or emails were less popular.	Expand both the number and variety of channels available and strengthen efforts to inform affected communities of the mechanisms. As the mobile network coverage has improved since the end of the project, consider establishing a hotline as the primary CFM channel and ensure all mechanisms are clearly communicated through multiple outreach formats, including FAQs. Regularly audit the use of each channel to ensure access, safety and inclusion.
Referrals	The CCS joint effort on service mapping and establishing referral pathways ran in parallel to implementation of the program. CORE and NIDAA actioned limited service mapping and activated known, albeit limited referrals, primarily for health needs.	Ensure that services are continuously mapped, referral pathways are established and operationalized in close coordination with CCS and inter-agency clusters. Moving forward, it is important to integrate referrals as part of the project design with dedicated funds. This will enable recipients to access complementary services based on identified needs that cannot be addressed through the program.
Humanitarian Principles	Close coordination with HAC throughout the CVA program cycle, particularly during targeting, helped CORE and NIDAA avoid significant delays in implementation. However, the blanket targeting and HAC approvals of cash recipient lists presents challenges to ensuring impartiality.	Continue to proactively seek HAC approvals and advocate, through the CCS, for a review of the new MPCA guidelines to collectively address the issue with HAC and government authorities, including the approval process for cash recipient lists. In addition, shifting from blanket to vulnerability-based targeting will strengthen adherence to the humanitarian principle of impartiality.
Local partner capacity and coordination	One of NIDAA's many value adds to the program and to CCS was their feedback on the PDM tool, which ensured that questions were culturally and conflict sensitive to uphold the "do no harm" principle. Uptake of this feedback took some time, as it required the CCS to update joint PDM tools in CommCare through the buy-in of other CCS members. This caused slight delays but enhanced program quality. CORE	Walk the talk on localization. Ensure meaningful engagement, equitable partnerships, and coordination that center local organizations' capacity and expertise. This should include more active involvement of local organizations across the entire project cycle, including project design, budgeting, and participation in the governance and technical structures of the CCS. CORE and NIDAA's partnership serves

	coordinated closely with NIDAA throughout the CVA program cycle through weekly check-ins and ad-hoc meetings as needed to track progress and jointly troubleshoot implementation issues.	as a blueprint for other INGOs collaborating with local actors.
MEAL	Registration included a baseline sample to reduce burden on communities and data collectors, especially with more than 500 recipients. Training can be targeted to data entry personnel, and CommCare access was centralized within CCS.	Consider adjusting the CCS MEAL plan so there is one user administrator per CCS partner. Define a representative sample to collect baseline data for a subset of households and registration-only data for the remaining families.
Gender and social inclusion	While MPCA significantly helped MPCA recipients meet basic needs, younger and male recipients were more likely to report improved well-being outcomes. In contrast, older and female participants continued to rely more heavily on emergency coping strategies. Spending patterns also reflected gender and age differences, for example, women spent more on childcare than male respondents.	Monitor and analyze well-being outcomes and coping strategies disaggregated by age, gender, and disability to continuously adapt programming toward more inclusive and impactful outcomes. Use the Washington Group Short Set of Questions within CCS tools to better understand needs and impact for persons with disabilities. Continue targeting women with MPCA whenever feasible and safe for recipients. Gender parity among registration teams is an important enabling factor for effectively and safely registering and targeting women.
Do no harm	The majority of respondents reported feeling safe. However, around one-fifth of PDM survey respondents reported feelings of unsafety, particularly during registration. There remains a gap in information and understanding regarding the types of risks program recipients may have faced in relation to MPCA and how these potential risks could be mitigated through program design and service delivery. No incidents were reported during the registration or distribution processes, nor mentioned during the PDM that suggested unsafe programing.	Continued active engagement with Community Committees remains crucial to inform real-time security analysis and adaptative planning. Moving forward, it is important to strengthen the operationalization of d*o no harm* principles in the program design and implementation is recommended. This includes ensuring that safety concerns and risks to program participants are adequately understood and mitigated within program design and MPCA delivery, including the inclusion of questions on safety and risks during community consultations.

7. Conclusions

As Sudan continues to be devastated by the ongoing conflict, MPCA remains essential to saving lives, preventing the collapse of market systems, reducing the reliance of vulnerable households on risky coping mechanisms, and protecting household assets. This pervasive humanitarian crisis is set against the backdrop of a protracted economic crisis, with the revised 2025 Humanitarian Response Plan (HRP) reporting that about 25 million (about 50% of the population) need humanitarian assistance and protection as the devasting conflict threatens to consume the entire country.

Consistent linkages between basic needs programming, sustainable livelihoods, income-generating activities, and recovery programming are more important than ever to ensure self-reliance and reduce dependence on humanitarian assistance. CORE and NIDAA's success reaching the hardest-to-reach communities in Khartoum, in the epicenter of the conflict, was underpinned by an equitable partnership that leveraged and strengthened the capacity of local organizations in assessments, coordination with key stakeholders (including FSPs), and harmonization efforts. CORE and NIDAA's partnership serves as a blueprint for other INGOs collaborating with local organizations.

The scale up of MPCA remains an essential strategy to save lives across Sudan. Moving forward, to enhance the program model, future interventions should: strengthen linkages between MPCA and livelihoods programming to support self-reliance; support recipients in opening bank accounts to promote financial inclusion and reduce reliance on proxies; and ensure ongoing gender and protection mainstreaming to mitigate unintended consequences and promote social inclusion and gender equality.

8. List of References

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